

ABRIDGE

STRATEGIC VALUE ASSESSMENT

Mayo Clinic

A framework for understanding the economics of ambient clinical documentation in inpatient settings

\$-135K

Projected Net Annual Value

50 hospitalists · 20,000 encounters · Inpatient

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Prepared for Brad Larmie

THE THESIS

Two Sources of Value

Most ROI models ask: "How much will you save?" We ask: "If we give hospitalists time back, what happens to your hospital?"

The answer reveals your strategy. There are two fundamental ways inpatient documentation time creates value:

CAPACITY UNLOCKED**\$0**

Hours returned to rounding, patient care, and throughput. Beds freed by earlier discharges. Admits processed faster.

The constraint is time. Remove it.**REVENUE OPTIMIZED****\$0**

Better documentation, higher CMI, reduced denials. Every encounter captured completely, coded correctly.

The notes drive the revenue.**STRATEGIC OBSERVATION**

Your model emphasizes revenue optimization—better documentation, improved coding. This is the playbook of hospitals seeking margin improvement.

FINANCIAL FRAMEWORK

The Investment Case

ROI calculations are necessary but insufficient. They answer 'Is this worth it?' but not 'How should we think about it?'

The right frame: ambient documentation is infrastructure, not an expense. It creates capacity that compounds.

PERIOD	VALUE	INVESTMENT	NET VALUE	CUMULATIVE
Year 1	\$0	\$135K	\$-135K	\$-135K
Year 2	\$0	\$135K	\$-135K	\$-270K
Year 3	\$0	\$135K	\$-135K	\$-405K

ECONOMICS INSIGHT

Notice the asymmetry: investment stays flat while value grows. This is the signature of infrastructure—fixed cost, scaling returns. By Year 3, you're generating \$0.00 for every \$1 invested. That's not a line item to be cut in a downturn. It's a competitive advantage to be protected.

CURRENT MODEL**\$705K/yr**

50 hospitalists · 70% utilization

FULL SCALE POTENTIAL**\$8.1M/yr**

500 hospitalists · 80% utilization

Per-hospitalist economics remain consistent at scale. Operational learning often improves them.

MODEL TRANSPARENCY

Every Number Has a Source

We don't hide assumptions. We highlight them. If you disagree with an input, change it. The model adapts. This isn't a black box designed to produce a predetermined answer. It's a thinking tool designed to stress-test scenarios.

YOUR INPUTS

hospitalists 50	Annual Encounters 20,000	Utilization 70%
Time Saved 2.5 min	Hours Returned 583	Investment \$135K/yr

REALIZATION RATES

RATE	VALUE	WHY THIS CONSERVATIVE
Throughput realization	15%	Operational coordination required
LOS impact	20%	Multi-factorial LOS drivers
Readmission reduction	10%	Documentation is one of many factors
Retention impact	4%	Multi-factor burnout attribution

METHODOLOGY NOTE

Our philosophy: conservative inputs, transparent logic. We'd rather you be pleasantly surprised than disappointed. These realization rates are based on observed implementations, not theoretical maximums.

NEXT STEPS

How to Use This Document

This isn't a sales document. It's a decision-support tool. Here's how to get the most from it:

1.

Stress Test the Inputs

What if utilization is lower than projected? What if time savings are more modest? Run scenarios. The value of this model is in its flexibility, not its point estimate.

2.

Scenario Plan

Map the pilots and rollouts. Which service lines first? Which hospitalist groups are most likely to adopt? Build the sequencing logic before the implementation plan.

3.

Track Outcomes

Pick 3 metrics to track post-implementation:

- Documentation time per encounter (target: -50%)
- Hospitalist satisfaction score (target: +15 pts)
- One operational metric aligned to your value drivers

The goal isn't to predict the future with precision. It's to give you a framework for making decisions with confidence—and adjusting as reality unfolds.